BASELINE → TREATMENT → FOLLOW-UP

Your testing and review plan

A reference cycle is not a personalised calendar. Tests listed as symptom-triggered or individual are not presented as mandatory tests at every visit. No universal imaging or tumour-marker interval is invented.

Check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Safety tests and symptoms	Blood count, kidney and liver function, glucose and thyroid tests. Review autoimmune disease, previous immune reactions, transplantation, infection, pregnancy possibility and all medicines, including steroids.	Blood count, kidney/liver tests, glucose and thyroid tests before every cycle, plus a careful symptom review. Cortisol and other tests are added when symptoms suggest endocrine or other immune effects. Normal routine blood results do not exclude colitis, lung, nerve or other inflammation.	Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy.	Immune toxicity can start or persist after the four-dose course. Keep the alert card, follow-up appointments, steroid-taper or hormone-replacement plan and the individual scan schedule. Report new symptoms after treatment has ended.	Do not restart held treatment until the team confirms the medicine, dose and timing. Safety tests and unresolved symptoms are reviewed first.
Scans and tumour results	Keep the pathology, stage and relevant biomarker report.	Response scans follow the individual disease plan, separate from safety bloods.	Report new symptoms rather than waiting for a scan.	Ask which scan, its planned date and who will explain the result.	A treatment hold or stop may change the next scan or follow-up date.
Written plan and contacts	Record actual oncology and out-of-hours numbers; examples are not working contact details.	Bring your medicine list and record the dose actually given or taken.	Keep the wallet card and written urgent-symptom plan available.	Ask for a printed updated plan when treatment changes.	Keep the last-dose date and any steroid, hormone or other follow-up instructions.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00105 v7: current regimen source

THE WHOLE TREATMENT JOURNEY

Ipilimumab alone — advanced melanoma: treatment and checks

Advanced melanoma

Ipilimumab by intravenous (IV) infusion once every 21 days, up to four doses. Complete within 16 weeks of the first dose, whichever limit comes first.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Safety tests and symptoms NCCP 00105	Blood count, kidney and liver function, glucose and thyroid tests. Review autoimmune disease, previous immune reactions, transplantation, infection, pregnancy possibility and all medicines, including steroids.	Blood count, kidney/liver tests, glucose and thyroid tests before every cycle, plus a careful symptom review. Cortisol and other tests are added when symptoms suggest endocrine or other immune effects. Normal routine blood results do not exclude colitis, lung, nerve or other inflammation.	Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy.	Immune toxicity can start or persist after the four-dose course. Keep the alert card, follow-up appointments, steroid-taper or hormone-replacement plan and the individual scan schedule. Report new symptoms after treatment has ended.	Do not restart held treatment until the team confirms the medicine, dose and timing. Safety tests and unresolved symptoms are reviewed first.
Scans and tumour results NCCP 00105	Keep the pathology, stage and relevant biomarker report.	Response scans follow the individual disease plan, separate from safety bloods.	Report new symptoms rather than waiting for a scan.	Ask which scan, its planned date and who will explain the result.	A treatment hold or stop may change the next scan or follow-up date.
Written plan and contacts NCCP 00105	Record actual oncology and out-of-hours numbers; examples are not working contact details.	Bring your medicine list and record the dose actually given or taken.	Keep the wallet card and written urgent-symptom plan available.	Ask for a printed updated plan when treatment changes.	Keep the last-dose date and any steroid, hormone or other follow-up instructions.

What each check is for

1

Treatment visits

21-day reference cycle

To help immune cells control advanced melanoma using ipilimumab alone; the planned infusion course is limited, but immune effects and follow-up continue beyond it.

Ask / **act:** Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00105

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-06 · Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.42